

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction for the supply of Wegovy (semaglutide) tablets for the treatment of Obesity / Weight Management

Summary of NICE / NICE CKS Guidelines for Obesity (Weight Management in Adults)

Overview

- Obesity is defined as excess body fat that presents a risk to health.
- Diagnosis is based on Body Mass Index (BMI), with waist circumference used to assess central fat distribution (especially in people with BMI <35).

Classification (Adults)

BMI (kg/m²)	Classification
18.5–24.9	Healthy weight
25–29.9	Overweight
30–34.9	Obesity Class I
35–39.9	Obesity Class II
≥40	Obesity Class III (severe/morbid)

Adjust BMI thresholds for South Asian, Black African, and some other minority ethnic groups (e.g. action at BMI ≥23 for overweight, ≥27.5 for obesity).

Assessment

- Measure: Weight, height, BMI, and waist circumference.
- Assess comorbidities (type 2 diabetes, hypertension, sleep apnoea, CVD), lifestyle factors (diet, activity, sleep, smoking, alcohol), readiness to change and previous weight loss efforts, and medications contributing to weight gain.

Management Approach (Stepwise)

1. Lifestyle Intervention (First-Line for All)

- Multicomponent programme: calorie deficit (~600 kcal/day deficit or 1200–1500 kcal/day), ≥150 minutes/week moderate-intensity activity, behavioural strategies (goal-setting, self-monitoring, problem-solving).
- Group-based programmes are often more effective.

2. Pharmacological Treatment

- Consider only if BMI ≥ 30 (or ≥ 27 with comorbidities) and lifestyle measures have been tried.
- This PGD covers Wegovy Tablets for weight management; see separate PGDs for other pharmacological options where authorised.
- Monitor side effects and response. Reassess at defined milestones.

3. Specialist Referral / Tier 3 Services

- Consider referral if BMI ≥ 40 , or ≥ 35 with comorbidities; or failure of Tier 2 interventions; or consideration of additional pharmacological or surgical options.

4. Bariatric Surgery (Tier 4)

- Consider for BMI ≥ 40 , or ≥ 35 with comorbidities, after failure of non-surgical options. Patient must be committed to long-term follow-up.

Follow-Up

- Regularly monitor weight, BMI, waist circumference, comorbidities.
- Support ongoing lifestyle changes.
- Relapse is common — plan for long-term support.

Patient Group Direction
for the supply/administration of
Wegovy (semaglutide) tablets
for the treatment of
Obesity / Weight Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC. • Pharmacy technician registered and practicing with the GPhC.
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SmPC for the products as well as BNF advice and any applicable national guidelines. • All users must previously have used PGDs to supply medication. • Must work in compliance with the SOPs of their own employer. • All users must have appropriate indemnity in place to cover this service.
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal. • All users must be up to date with MHRA and safety alerts with regards to medical products. • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Wegovy (semaglutide) tablets are a UK-licensed GLP-1 receptor agonist indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management, including weight loss and weight maintenance, in adults with an initial BMI ≥ 30 kg/m² (obesity), or ≥ 27 kg/m² to < 30 kg/m² (overweight) in the presence of at least one weight-
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	related comorbidity (e.g. hypertension, dyslipidaemia, obstructive sleep apnoea, cardiovascular disease, type 2 diabetes). This medicinal product is subject to additional monitoring (black triangle) — report all suspected adverse reactions via the MHRA Yellow Card scheme.
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Initial Assessment

Before commencing treatment with weight management therapies, the patient must be assessed by a registered healthcare professional. The initial assessment should be face-to-face.

The assessment should include, but may not be limited to:

- Causes of weight gain — the patient should be referred to their GP if their prescribed medication is causing weight gain.
- Patient's lifestyle, diet and level of exercise.
- Has the patient tried to lose weight in the past? Discuss measures that may have been successful or unsuccessful.
- Explore other factors that may be causing weight gain, such as mental health, environmental and psychological factors. Consider signposting to another health professional if needed, e.g. a mental health team.
- Does the patient have other disease states that may pre-dispose them to weight gain? Consider referring to a GP for further advice.
- What are the patient's expectations in terms of weight loss? Are the expectations realistic for their height and body frame?
- Calculate Body Mass Index (BMI) and determine the patient's ideal weight. Set a realistic target weight with the patient and agree review intervals.

Inclusion criteria	Adults aged 18 years and over. BMI ≥ 30 kg/m ² , OR BMI ≥ 27 to < 30 kg/m ² with at least one weight-related comorbidity. Valid informed consent given. Patient willing to commit to a reduced-calorie diet and increased physical activity. Patient able to follow the empty-stomach administration requirements (≥ 8 -hour fast; 30-minute wait).
Exclusion criteria	Under 18 years of age. Hypersensitivity to semaglutide or any excipient. Pregnancy, pregnancy planned within 2 months, or breastfeeding. Type 1 diabetes. History of pancreatitis. Severe gastroparesis. Active eating disorder. Severe renal impairment (eGFR < 30 ml/min/1.73 m ²), including patients with end-stage renal disease.

	Severe hepatic impairment. Diabetic retinopathy (refer for specialist review). Personal or family history of medullary thyroid carcinoma or MEN 2 syndrome. Concurrent use of another GLP-1 or GIP receptor agonist.
Cautions	NAION (non-arteritic anterior ischaemic optic neuropathy): advise patients to seek urgent review for any sudden visual loss; discontinue if NAION confirmed. GI effects and dehydration — counsel on fluid intake, especially with renal impairment. Delayed gastric emptying: consider aspiration risk before procedures under general anaesthesia or deep sedation. Acute pancreatitis has been reported in patients treated with GLP-1 receptor agonists, including reports of necrotising pancreatitis and reports with a fatal outcome. Patients should be informed of the symptoms of acute pancreatitis, including persistent, severe abdominal pain, and advised to seek immediate medical attention if they occur. If pancreatitis is suspected, semaglutide should be discontinued; if the diagnosis of pancreatitis is confirmed, semaglutide should not be restarted. In the absence of other signs and symptoms of acute pancreatitis, elevations in pancreatic enzymes alone are not predictive of acute pancreatitis. Hypoglycaemia — semaglutide lowers blood glucose and can cause hypoglycaemia. Patients should be aware of the risk of hypoglycaemia and be educated on the signs and symptoms of hypoglycaemia. Diabetic retinopathy — in patients with diabetic retinopathy treated with semaglutide, an increased risk of developing diabetic retinopathy complications has been observed. Rapid improvement in glucose control has been associated with a temporary worsening of diabetic retinopathy, but other mechanisms cannot be excluded. Patients with diabetic retinopathy using semaglutide should be monitored closely and treated according to clinical guidelines. There is no experience in patients with congestive heart failure. Hepatic impairment — use cautiously in patients with mild or moderate hepatic impairment. No dose adjustment is required based on age, but therapeutic experience in patients ≥ 85 years of age is limited. Women of childbearing potential are recommended to use contraception when treated with semaglutide. If a patient wishes to become pregnant, or pregnancy occurs, semaglutide should be discontinued. Semaglutide should be discontinued at least 2 months before a planned pregnancy due to the long half-life. Insulin or sulfonylurea users — hypoglycaemia risk; refer to the prescribing GP for dose review before supply. The risk of

	hypoglycaemia can be lowered by reducing the dose of sulfonylurea or insulin when initiating treatment with a GLP-1 receptor agonist. Levothyroxine — thyroxine exposure increases ~33%; recommend thyroid function monitoring via the GP. Warfarin/coumarins — recommend more frequent INR monitoring on initiation. Other oral medicines — absorption may be affected; take other oral medicines at least 30 minutes after Wegovy. Switching from Wegovy 2.4 mg injection: may transition to 25 mg tablets once daily, starting one week after the last injection. Dizziness may occur during dose escalation — caution with driving.
Actions if patient is excluded or declines treatment	• Discuss the reason for exclusion with the patient and ensure they understand. • Advise on alternative treatment options and how these can be accessed (e.g. GP, specialist weight management service, lifestyle programmes). • If the exclusion relates to an undiagnosed or unmanaged comorbidity, recommend GP review. • Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Wegovy (semaglutide) tablets: 1.5 mg, 4 mg, 9 mg and 25 mg. Black-triangle medicine (additional monitoring). Record product name and batch number for traceability.
Legal category	POM
Storage	• Store in the original blister package to protect from moisture and light. No special temperature storage conditions required (per SmPC 6.4).
Route/method of administration	• Oral, once daily. Take on an empty stomach after a fasting period of at least 8 hours. Swallow whole with a sip of water (up to half a glass, ~120 ml). No liquid other than water should be used to aid swallowing the tablet. Do not split, crush or chew. Wait at least 30 minutes before the first food, drink or other oral medicinal products of the day. Adherence to the dosing regimen is recommended for optimal effect.
Dose and frequency	Start at 1.5 mg once daily for one month. Escalate monthly through 4 mg and 9 mg to the maintenance dose of 25 mg

	once daily, with a minimum of one month at each step; if needed, the dose can be maintained at the previous level. Maximum 25 mg once daily. Only ONE tablet per day — never combine tablets to approximate a higher dose. Missed dose: skip it and take the next dose the following day. Switching from semaglutide 2.4 mg injection: 25 mg once daily, starting one week after the last injection.
Quantity to be administered and/or supplied	One calendar pack (30 tablets) of the appropriate strength per supply during titration; maintenance supplies per the treatment plan and review interval.
Maximum or minimum treatment period	Review weight, tolerance and adherence before each dose escalation and at every supply. Evaluate response on the maximum tolerated dose; discontinue if weight loss is < 5% of initial body weight after 16 weeks at that dose. No fixed maximum duration while criteria remain met and benefit continues.
Adverse effects	Very common ($\geq 1/10$): - Gastrointestinal: nausea, diarrhoea, vomiting, constipation. - Headache, fatigue. Common ($\geq 1/100$ to $< 1/10$): - Abdominal pain, dyspepsia, eructation, flatulence, abdominal distension, GORD. - Dizziness, hypoglycaemia (in patients with type 2 diabetes). - Cholelithiasis, gallstones. - Hair loss (alopecia). Uncommon and rare: - Acute pancreatitis. - Diabetic retinopathy complications in patients with type 2 diabetes. - Hypersensitivity reactions including anaphylaxis. - Acute kidney injury (typically secondary to dehydration). Dysaesthesia: - Skin sensations such as tingling, burning, and numbness have been reported. These sensations can be manageable and may improve over time. If severe, the patient should decrease their dose. Counsel patient on warning symptoms (especially severe abdominal pain, severe vomiting and dehydration, jaundice, vision changes). Refer to current SmPC for full safety information.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • That valid informed consent was given. • Name of individual, address, date of birth and GP. • Name of healthcare practitioner. • Name and brand of medication. • Date of supply, dose, form, route and quantity supplied. • Advice given, including advice given if excluded or declines treatment. • Details of any adverse drug reactions and actions taken. Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over — keep records for 8 years. • Children aged under 18 years — keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday).

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication. Provide written information on important administration guidance, missed-dose rules, side-effect counselling and emergency contact details.
Follow-up advice to be given to patient or carer	• Seek medical advice if symptoms worsen rapidly or significantly, the patient is unable to tolerate the medication, or they become systemically very unwell. • Adhere to the agreed lifestyle changes — diet, physical activity, sleep, alcohol and smoking — alongside the medication. • Report and seek urgent medical advice for severe abdominal pain (possible pancreatitis), persistent vomiting/dehydration, signs of gallbladder disease (RUQ pain, jaundice, fever), or symptoms suggestive

	of severe allergic reaction. • Attend all review appointments. Bring weight diary / measurements if requested. • Take on an empty stomach after at least 8 hours fasting; swallow whole with up to 120 ml of water; wait at least 30 minutes before food, drink or other oral medicines. • If you experience skin sensations such as tingling, burning or numbness, contact your pharmacist or prescriber — these symptoms often settle but persistent or severe symptoms may warrant a dose reduction. • If you have type 2 diabetes, attend retinal screening as advised — semaglutide can be associated with transient worsening of diabetic retinopathy in some patients.
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Key references

• NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017. https://www.nice.org.uk/guidance/mpg2 • NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017. • NICE CKS Guidance: Obesity. • British National Formulary (BNF). • Summary of Product Characteristics (SmPC) for the medicinal product.
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Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates

Name of healthcare professional	Job title	Registration number	Signature
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Valid from date	Expiry date
10/07/26	09/07/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		10/07/26
Chris Pilkington	Pharmacist		10/07/26

	GPhC: 2046322		
Jane Wilkins	Pharmacist (Clinical Lead) GPhC: 2043424		

Change history

Version number	Change details	Date
001	Development and issue of standalone Wegovy (semaglutide) tablets PGD, per UK SmPC (tablets 1.5/4/9/25 mg)	10th July 2026
002	Amendments following PPH clinical review (J. Tipping): tablet-specific wording throughout; expanded cautions per SmPC (pancreatitis, hypoglycaemia, diabetic retinopathy, congestive heart failure, hepatic impairment, age ≥ 85 , pregnancy/contraception); renal and hepatic exclusions clarified; referral wording softened to "consider"	23rd July 2026
003	Correction of clinical lead identity: Jane Wilkins, GPhC 2043424 (previously misrecorded as Janey Tipping, GPhC 2049827). GPhC register verified.	28th July 2026